

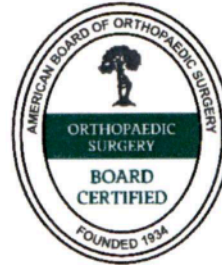
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ASSH | American Society
for Surgery of the Hand



COASTAL
MEDICAL GROUP
Orthopedic, Spine Surgeons
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RE: Martinez, Lelia
Date of Birth: 11/09/1948
Date of Injury: 02/29/2024
Date of Exam: 06/06/2024
Parts of Body: Right shoulder, Right wrist, Right hand

ORTHOPAEDIC SPECIALTY SURGEON/PHYSICIAN FOLLOW-UP VISIT/SUPPLEMENTAL REPORT

Ms. Lelia Martinez was seen for ongoing follow-up orthopedic specialty surgeon/physician examination, evaluation and management.

I reviewed the patient's health history as documented in the initial visit and it remains unchanged.

Prior and any additional medical records were reviewed.

Today's visit was completed as a telemedicine visit.

The following is a dictated narrative report based on pre-examination interview, specialty examination, review of diagnostic/imaging studies and review of medical records. Included in this report are findings, diagnosis/impression, prognosis, work status and further evaluation and treatment recommendations with respect to this injury.

Ms. Martinez returns to the office for ongoing orthopedic hand and upper extremity subspecialty follow-up examination, evaluation and management. I have been following the patient for diagnoses of:

1. Right shoulder: Sprain/strain/contusion, query internal derangement (specifically proximal long head of biceps pathology and/or rotator cuff tear).
2. Right upper extremity: Paresthesias (query peripheral entrapment neuropathy such as carpal tunnel syndrome and/or cervical radiculopathy).

It was again my pleasure to see Ms. Martinez to assess any progress the injured patient has made with regard to the above diagnoses.

Interim History/Current Complaints

Since the last visit, Ms. Martinez reports her right shoulder is improving, but she has continued pain with overhead activity and placing the hand behind the back. She reports physical therapy of the right shoulder has been helpful and she has improved lifting strength. She would like to continue with physical therapy and avoid injections and surgery per her report.

Review of Systems

The review of systems was all negative for a 14-point review of systems except as documented on the initial visit and those symptoms associated with the injury/illness.

Physical Examination (Date of Exam: 04/04/2024)

Focused Examination:

Focused examination of the left shoulder shows full active range of motion with no provocative maneuvers.

Focused examination of the right shoulder shows active range of motion, forward flexion of 85 degrees, abduction 85 degrees, abduction external rotation 60 degrees, external rotation at the side 40 degrees. She has 4/5 strength with resisted external rotation strength and 5-/5 with resisted internal rotation strength at the side. Positive provocative maneuvers include positive Speed, Jobe, cross body adduction, bear hug maneuver. She has tenderness to palpation at the long head of biceps tendon sheath and AC joint.

Focused examination of the right wrist shows full active range of motion with positive Durkan, Tinel and Phalen maneuvers along the carpal canal.

Focused examination of the right hand shows diminished sensation with light touch testing along the middle finger and ring finger with 5/5 interossei and abductor pollicis brevis strength. She has full composite flexion, extension, and opposition of all digits of the right hand.

Diagnostic Studies

X-rays of the right shoulder including 3 views performed on 05/28/2024 as reviewed by radiologist, Edward Rothenberg, M.D., at Imaging Healthcare specialists shows no acute fracture or dislocation and only mild degenerative changes of the AC joint and degenerative enthesophyte of the acromion versus os acromiale and amorphous calcification adjacent to the humeral head, which may represent calcific tendinosis of the rotator cuff versus intra-articular loose body. Per my independent review and interpretation of these imaging studies, I agree with the radiologist's report.

MRI of the right shoulder without contrast performed on 04/27/2024 at Imaging Healthcare specialist as reviewed by radiologist, James Cooper, M.D., shows large full-thickness tear involving the entire supraspinatus tendon and anterior fibers of the infraspinatus tendon with tendon retraction to the superior glenoid and moderate fatty atrophy; subscapularis tendon high-grade tear (75% to 90%); long head of biceps tendinosis, which is moderate-to-severe of the intra-articular segment; moderate sized glenohumeral joint effusion with 9 x 4 mm loose body in the posterior aspect of the axillary recess; AC joint osteoarthritis, which is mild-to-moderate; large os acromiale with large effusion in the synchondrosis between the acromion and os acromiale. Per my independent review and interpretation of these imaging studies, I agree with the radiologist's report.

Electrodiagnostic studies of the right upper extremity completed on 05/30/2024 by Robert Scott, M.D., show no peripheral nerve injuries or disorders detected in the right upper limb including median, radial or ulnar neuropathies and no evidence of right cervical motor radiculopathy.

Impression/Diagnoses

During this follow-up examination, evaluation and management encounter, a discussion was held with the patient in which the diagnoses were again explained in layman's terms. It is my opinion that the patient suffers from:

1. Right shoulder: Sprain/strain/contusion, rotator cuff tear (entire supraspinatus tendon and anterior fibers of the infraspinatus tendon, which is a large full-thickness tear with tendon retraction to the superior glenoid and moderate fatty atrophy as well as subscapularis high-grade 75% to 90% tear), long head of biceps tendinosis (moderate-to-severe of the intra-articular portion), loose body (9 x 4 mm in the posterior aspect of the glenohumeral joint axillary recess), AC joint osteoarthritis (mild-to-

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moderate) aggravation, large os acromiale with large effusion in the synchondrosis between the acromion and the os acromiale.

2. Right upper extremity: Paresthesias (electrodiagnostic studies negative).

Recommendations

The treating recommendations have been discussed and/or communicated with the patient as outlined within this transcribed dictation. During this follow-up examination, evaluation and management encounter, a discussion was held with the patient in which the above diagnoses along with the therapeutic options/alternatives were explained in detailed layman's terms.

She is to continue with physical therapy of the right shoulder at a frequency of 2 times per week for a duration of 6 weeks and is to follow up in 8 weeks for routine follow-up care. Consideration for targeted corticosteroid injections will be discussed in the future with the patient. Additionally, she may meet eventual indications for surgical arthroscopy of the right shoulder for diagnostic and therapeutic purposes inclusive of rotator cuff repair attempt and if arthroscopic surgery were to be unsuccessful, she could be staged with a right shoulder reverse total shoulder arthroplasty 9-12 months later after the right shoulder arthroscopic surgery as surgical considerations in her case.

The risks, benefits and alternatives to the above treatment plan were explained in detail to the patient who vocalized understanding. All questions were answered to the patient's satisfaction. No guarantees were made with any of my treatment nor are they ever and the patient understood this as well.

Follow-Up Appointment

8 weeks.

Reimbursable Services

Evaluation & Management (E&M):

My evaluation and management service for date of service 06/06/2024 is a separately reimbursable service.

Dictation Services:

Preparation time for dictation of this report, time spent performing dictation and time spent reviewing/editing/signing the transcribed dictation required an additional 15 minutes of my time. This time spent performing this task is also a separately reimbursable service.

Sincerely,



Kristopher L. Downing, M.D.
Orthopaedic Surgeon
Hand and Upper Extremity Surgeon & Microsurgeon

KLD/TDYNE/bmb.XG

Electronically signed by Kristopher L. Downing, MD on 06/11/2024 07:54:52 AM